

PROJECT NARRATIVE

Strengthening Global Health Security by improving public health capacity to detect, notify, and respond to disease outbreaks globally

Opportunity number: CDC-RFA-JG-26-0056

Assistance Listing: 93.318

FAIRBANKS MEDICAL CENTRE LIMITED

Uganda-registered local partner · Kampala

Your health, our mission.

Year 1 federal request: USD 7,500,000

Component 1 (core): USD 3,000,000 · Components 2–5 (ABU): USD 4,500,000

Primary country: Uganda · Period of performance: five 12-month budget periods

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Acronym list

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ABU — Approved but unfunded

AAR — After-action review

CHW — Community Health Worker

DGHP — Division of Global Health Protection

EPMP — Evaluation and Performance Measurement Plan

FCHIP — FairBanks Community Health Intelligence Platform

GHS — Global Health Security

IPC — Infection prevention and control

KCCA — Kampala Capital City Authority

MoH — Ministry of Health

NISS — National Integrated Surveillance System

NOFO — Notice of Funding Opportunity

PHE — Public health emergency

VHT — Village Health Team

7-1-7 — Detect within 7 days, notify within 1 day, respond within 7 days

Background and approach

Problem and context

FAIRBANKS MEDICAL CENTRE LIMITED (FairBanks) is a Uganda-registered health organisation in Kampala. We operate a licensed medical centre and FairBanks Community Reach with Community Health Workers (CHWs) and Village Health Teams (VHTs) in Bukoto, Kyebando, Kisaasi, Kamwokya, Kikaaya, and nearby peri-urban communities. Our slogan is Your health, our mission.

Uganda faces repeated infectious disease threats shaped by geography, rapid urban growth, dense peri-urban settlements, and high population movement along East African corridors. Outbreaks that start in communities can grow before national systems see a clear signal. Last-mile gaps remain practical and urgent: community reports, facility encounters, and laboratory referrals often sit in separate paper or digital silos; district and national teams receive incomplete or delayed information; and surge capacity at subnational level is uneven when caseloads rise.

In FairBanks catchments, CHWs and VHTs already visit homes and schools, and the Medical Centre sees patients every day. Too often, those signals do not reach formal surveillance pathways quickly enough for early action. Uganda's progress on global health security, including integrated surveillance such as the National Integrated Surveillance System (NISS), creates a clear opportunity: strengthen the community-to-facility-to-district link so early warning reaches decision-makers before a local cluster becomes a wider emergency.

This cooperative agreement will not fund routine clinical care. FairBanks will continue clinic operations on its own resources. Award funds will strengthen surveillance, data quality, workforce readiness, referral follow-up, and coordination under Ministry of Health (MoH) leadership — helping communities recognise threats earlier, connect signals into public systems, act in time, and contain risk closer to where people live.

Evidence-based activities and technical approach

Our approach is grounded in FairBanks Community Reach — the operating cascade from community members to CHWs/VHTs, Community Reach programmes, the Medical Centre, research and skills, and economic empowerment — with a continuous Data and Feedback loop. The FairBanks Community Health Intelligence Platform (FCHIP) is the intelligence component on that loop. FCHIP supports structured mobile capture (including offline use), supervisory review, simple GIS views, and MoH/NISS-aligned exports. It is designed to feed government systems, not replace them.

Year 1 activities are evidence-informed, realistic for Uganda peri-urban and district settings, and measurable. We apply time-bound practice consistent with 7-1-7 principles: detect

community and facility signals sooner, notify through agreed channels within one day of confirmation thresholds, and support early response within a defined window. We combine: (1) CHW/VHT structured reporting; (2) facility flag and referral tracking; (3) weekly data quality checks; (4) district dashboards and joint reviews; (5) workforce training and surge rosters with One Health awareness; and (6) contingency playbooks for Components 2–5 when CDC activates emergency funding.

Complementing ongoing response

FairBanks will build on, not duplicate, MoH, district/KCCA, and other CDC-funded partner work. We will map existing surveillance and response actors in Year 1 Quarter 1, share work plans with MoH and CDC activity managers, and adjust geography or tasks if DGHP priorities change. Laboratory surge and selected specialised modules will be delivered with MoH-aligned partners. Software, standard operating procedures (SOPs), and training materials developed under the award will be available to MoH and CDC for appropriate use.

Methodology by NOFO strategy

FairBanks will lead Strategies 1, 2, and 5 in Year 1, and support Strategies 3, 4, 6, and 7 with MoH-aligned partners. The table summarises the Year 1 method.

Strategy	FairBanks Year 1 method
1. Surveillance / systems with governments	FCHIP capture; data quality rules; MoH/NISS-aligned exports; simple GIS early warning.
2. Interoperable public health data	Integrate community, facility, and lab signals; weekly reporting; district dashboards.
3. Public health programmes / service links	Community-facility links for priority diseases (e.g., malaria/fever clusters, VHFs, mpox, TB, HIV, immunisation campaigns) under MoH guidance.
4. Assessment and corrective action	Joint district reviews; after-action reviews (AARs) after drills or real events; continuous improvement.
5. Workforce	CHW/VHT and facility training; supervised practice; surge roster; One Health-aware sessions.
6. Networks and surge connect	Partner coordination; MoH/CDC information sharing; activation pathways for Components 2–5.
7. Small/large outbreak response	Components 2–5 contingency work plans and budgets for moderate/substantial PHE and emerging threats.

Year 1 Component 1 work plan (Core GHS — USD 3,000,000)

Component 1 is expected initial funding. It strengthens last-mile detection, notification, and early response under MoH leadership.

Quarter	Key activities and milestones
Q1	Staffing and onboarding; MoH/district kickoff; CHW/VHT cohort mapping; FCHIP form finalisation; baseline 7-1-7 timing measure; partner non-duplication map; first supervisory visits.
Q2	Weekly community reporting live in primary catchments; facility flag and referral tools in use; first MoH/NISS-aligned export test; training wave 1 (surveillance and One Health awareness); district dashboard prototype.
Q3	Expand supervised CHW/VHT coverage; second export/interoperability test; joint district review; surge roster exercise / drill 1 with AAR; strengthen referral outcome documentation for priority signals.
Q4	Consolidate data quality targets; training wave 2; drill or real-event AAR 2; handover package of SOPs/tools for MoH/CDC use; Year 1 performance report; continuation planning.

Components 2–5 contingency work plans (ABU)

Components 2–5 remain approved but unfunded until CDC activates emergency or emerging-threat funding. Each has a scaled Year 1 budget and a ready activation sequence under MoH direction. FairBanks will not draw these funds without CDC activation and agreed tasking.

Component	Year 1 ask	Activation focus
2. Small-scale outbreak / PHE	USD 1,000,000	Moderate surge: rapid community investigation, referral surge, refresher training, temporary staffing and supplies in affected catchments/districts.
3. Large-scale outbreak / PHE	USD 1,500,000	Substantial surge: multi-district coordination, expanded CHW/VHT surge, heavier contractual logistics, intensified reporting to MoH/CDC.
4. Small emerging threats	USD 1,000,000	Enhanced surveillance, risk communication, targeted screening, and rapid community-facility links for an emerging threat at limited scale.
5. Large emerging threats	USD 1,000,000	Broader geographic coverage and intensified field/contractual surge for a large emerging threat under MoH/CDC activation (SF-424A Form 2).

Activation sequence (all ABU components): (1) CDC/MoH activation notice; (2) 48–72 hour internal surge stand-up; (3) deploy rostered supervisors and CHWs/VHTs; (4) open daily situation reporting; (5) execute investigation, referral, risk communication, and logistics tasks; (6) AAR and cost reporting within agreed timelines.

Countries and delivery location

Year 1 primary country: Uganda. FairBanks is incorporated and operating in Uganda (Company No. 80020003843337; TIN 1053370026; NSSF NS043295), with principal place of business in Kampala and public community base at Kyebando-Kisalosalo, Northern Bypass, Kampala.

Regional learning: East Africa corridor learning and cross-border signal protocols under MoH Uganda leadership. We do not claim unregistered field operations outside Uganda. Additional countries will be included only with documented partner letters and registration (CONFIRM before naming).

Anticipated outcomes

- Faster detection and response using 7-1-7 style timing from community signal to district notification.
- Stronger weekly community and facility reporting, with successful MoH/NISS-aligned export tests.
- Trained CHWs/VHTs and an exercised surge roster.
- Improved referral and follow-up documentation for priority disease signals.
- Practical tools, SOPs, and learning available for MoH and CDC use.
- Contingency capacity (Components 2–5) ready for activation without creating parallel systems.

Evaluation and performance measurement plan

FairBanks will implement an Evaluation and Performance Measurement Plan (EPMP) aligned to this NOFO's logic-model outcomes and DGHP FY26 partner-level indicators (surveillance, community mitigation, emergency operations, IPC, and laboratory support as applicable). About 5–10% of funds support monitoring, evaluation, and learning. A full detailed EPMP will be finalised with CDC within six months of award, as required.

Performance measures and reporting

Year 1 project measures (draft targets — to be confirmed with CDC after award):

Measure	Year 1 target	Frequency
Median days from community signal to district notification	Baseline in Q1; improve toward 7-1-7 by Q4	Monthly / quarterly
Active CHWs/VHTs with complete weekly reports	At least 80% by Q4	Weekly / quarterly
Priority disease referrals documented with outcome	At least 70%	Monthly / quarterly
Successful MoH/NISS-aligned export tests	At least 2	By event / quarterly
After-action reviews (drills or real events)	At least 2	After each event
Training cohorts completed (CHW/VHT and frontline)	At least 2 waves	Quarterly

Each programme milestone in the Component 1 work plan and each ABU activation will have linked indicators in financial and programmatic reports. Indicator definitions, numerators/denominators, and data sources will be locked with CDC in the detailed EPMP.

Use of findings and dissemination

FairBanks will use evaluation findings and performance data to: (1) coach CHW/VHT supervisors; (2) fix data-quality gaps; (3) rebalance outreach and referral support; (4) improve SOPs and training; and (5) brief MoH, districts/KCCA, and CDC on what is working. Results will be disseminated through quarterly review meetings, written reports, AARs, and shared dashboards — demonstrating the value of last-mile surveillance that feeds government systems.

Financial and programmatic reporting system

FairBanks will operate a grants finance and programme tracking system able to generate: (1) quarterly financial reports showing fund disbursement by component and object class; (2) quarterly programmatic reports with indicator tables and narrative progress; (3) activation-specific burn and output reports for Components 2–5 if funded; and (4) annual

performance / continuation inputs as required. Segregation of duties, payment approvals, and documentation will support timely administration of personnel, grants, and contracts.

Data management plan

This initial Data Management Plan (DMP) describes how FairBanks will manage award data. A final DMP will be completed with CDC after award.

- Data types: CHW/VHT visit and symptom forms; facility flags; referral outcomes; training records; drill/event AARs; de-identified aggregate surveillance exports.
- Systems: FCHIP offline-capable mobile capture with secure sync; facility/programme dashboard; MoH-aligned export formats. No parallel patient silo replacing MoH systems.
- Privacy and security: consent-aware collection where required; role-based least-privilege access; anonymisation for analytics; encryption in transit for sync; audit logs for administrative access.
- Quality: validation rules at entry; supervisory review; monthly data-quality checks; documented corrections.
- Sharing and retention: MoH and CDC receive agreed exports and SOPs; retention follows award terms and applicable Uganda/CDC requirements.
- Ownership: tools and documentation developed under the award will be available to MoH and CDC for appropriate use.

Organizational capacity

FAIRBANKS MEDICAL CENTRE LIMITED is a Uganda company (No. 80020003843337; TIN 1053370026; NSSF NS043295) with principal place of business in Kampala. FairBanks brings a live Medical Centre and Pharmacy; active Community Reach with CHWs/VHTs; and a working FCHIP MVP (mobile capture, supervisory workflows, GIS roadmap, and a secure path for facility data exchange). Public community base: Kyebando-Kisalosalo, Northern Bypass, Kampala. Catchments include Bukoto, Kyebando, Kisaasi, Kamwokya, and Kikaaya.

Leadership: Racheal Nabukeera, Managing Director and Co-founder — more than 15 years in Uganda private health leadership; MA Social Sector Planning (Makerere); doctoral studies in progress; links through Uganda Healthcare Federation networks. Day-to-day award management will add a technical lead for surveillance, FCHIP/data manager, M&E specialist, finance and grants compliance manager, training coordinator, community surveillance officers, and CHW/VHT supervisors (see budget narrative and staffing attachments).

Local experience and institutional capacity. FairBanks already delivers community outreach, maternal and child health touchpoints, chronic-disease screening, school health education, and facility care in the same communities where surveillance signals arise. That combination — field relationships plus a clinical anchor — is the operational base for GHS last-mile work under MoH leadership. We will document in-country local partner preference criteria in the required attachment package.

Operational capacity in Uganda. FairBanks is locally registered, Luganda/English capable in community settings, and staffed for peri-urban Kampala operations. Multi-country expansion is not claimed for Year 1 field delivery; corridor learning stays under MoH Uganda. Additional countries require partner LOIs and registration before inclusion.

Financial and administrative capacity. FairBanks will strengthen federal award controls for this cooperative agreement: chart of accounts by component, procurement documentation, timesheets for effort reporting, bank reconciliations, and timely financial close. We are honest that FairBanks has not yet been prime on a multi-million CDC GHS award; the Year 1 plan therefore invests in finance compliance capacity, clear SOPs, and partner contracting discipline. Supporting attachments (experience statement; financial capability statement; resumes/JDs; organizational chart; locally registered organisation documents) will provide evidence for review.

Collaborations

If funded, FairBanks will collaborate with CDC programmes and CDC-funded organisations, and with organisations not funded by CDC, to avoid duplication and to leverage host-government leadership.

CDC programmes and CDC-funded organisations

- Coordinate with CDC Uganda / DGHP activity managers on work plans, indicators, and activation of Components 2–5.
- Share geographic and technical maps with other CDC-funded partners to prevent overlap in the same catchments.
- Participate in partner coordination meetings and information-sharing protocols set by CDC and MoH.

Host government and other stakeholders

- Ministry of Health — leadership for surveillance pathways, NISS-aligned exports, and surge tasking.
- District health teams and KCCA — weekly reporting, joint reviews, referral follow-up, and local surge coordination.
- Community structures — CHWs/VHTs, local leaders, schools, and community spaces for education and early warning.
- MoH-aligned laboratory and implementing partners — contractual support for diagnostics and specialised surge modules.
- Civil society, academia, and private providers — as needed for training, evaluation, or non-duplicative service links.

Collaboration principle: FairBanks feeds government systems and community response. We do not create a parallel surveillance authority. Tools and learning from the award remain available for MoH and CDC use.

End of project narrative. Supporting evidence is provided in separate attachments as required by the NOFO.